

1CD-0-3

carcinoma, infiltrating duct, NOS 8500/3

Site: breast, NOS 850.9 *hw* 5/24/11

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Notes		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	<i>MB</i>	Date Reviewed <i>5/24/11</i>

DIAGNOSIS

(A) SENTINEL LYMPH NODE #1, LEFT AXILLARY:

One lymph node, no tumor identified (0/1).

Immunohistochemical stains for cytokeratin are negative.

(B) LEFT BREAST AND LOW AXILLARY CONTENTS, MASTECTOMY:

TWO FOCI OF INVASIVE DUCTAL CARCINOMA OF BREAST, MODIFIED BLACK'S NUCLEAR GRADE 2-3 (INTERMEDIATE TO HIGH GRADE) LOCATED AT THE 1 O'CLOCK AND 2 O'CLOCK POSITIONS WHICH MEASURE 2.5 CM AND 1.5 CM IN GREATEST DIMENSION, RESPECTIVELY.

Angiolymphatic invasion is not identified.

EXTENSIVE DUCTAL CARCINOMA IN SITU (DCIS), MODIFIED BLACK'S NUCLEAR GRADE 3 (HIGH GRADE), SOLID AND CRIBRIFORM TYPES WITH NECROSIS AND ASSOCIATED MICROCALCIFICATIONS.

DCIS IS 3 MM FROM THE SUPERFICIAL MARGIN ADJACENT TO THE 1 O'CLOCK TUMOR.

INVASIVE CARCINOMA IS 6 MM FROM THE SUPERFICIAL MARGIN ADJACENT TO THE 1 O'CLOCK TUMOR.

INVASIVE CARCINOMA AND DCIS ARE PRESENT AT THE SUPERFICIAL MARGIN ADJACENT TO THE 2 O'CLOCK TUMOR.

DCIS IS LESS THAN 1 MM FROM THE DEEP MARGIN ADJACENT TO THE 2 O'CLOCK TUMOR.

INVASIVE CARCINOMA IS 2 MM FROM THE DEEP MARGIN ADJACENT TO THE 2 O'CLOCK TUMOR.

Five low axillary lymph nodes, no tumor identified (0/5).

Nipple, no tumor identified.

Skin with seborrheic keratosis.

SEE COMMENT.

(C) LYMPH NODE, ADDITIONAL LOW LEFT AXILLARY, EXCISION:

Three lymph nodes, no tumor identified (0/3).

(D) NEW SUPERIOR LATERAL MARGIN, EXCISION:

Skin and breast tissue, surgical resection margins negative for invasive and in situ carcinoma.

Seborrheic keratosis.

See comment.

(E) NEW SUPERFICIAL LATERAL MARGIN, EXCISION:

Fibroadipose tissue, no tumor identified.

See comment.

(F) NEW INFERIOR LATERAL MARGIN, EXCISION:

UUID: 88292748-E882-4155-A458-CF9F0E60B36E
TCGA-GM-A2DN-01A-PR



Small focus of an atypical ductal proliferation.

Surgical resection margin, no invasive or in situ carcinoma identified.

Skin, no tumor identified.

See comment.

Entire report and diagnosis completed by

COMMENT

In the left mastectomy specimen, there is extensive DCIS which is associated with both foci of invasive carcinoma. The DCIS involves an area of breast tissue which is estimated to measure 7 to 8 cm. The two foci of invasive carcinoma are separated by approximately 2.0 cm of breast tissue which has foci of DCIS. There is a morphologic spectrum of DCIS ranging from low grade to high grade with necrosis. Both invasive carcinomas have an associated prominent lymphocytic response. The nuclear grade of the invasive carcinoma ranges from intermediate to high grade.

The additional submitted margins and final margin status were discussed with Dr. The additional submitted margins (specimens D-F) appear to cover the superficial aspect of both tumors.

Specimen D has skin and breast tissue and the true surgical resection margins (superior/lateral aspect of the specimen) are negative for invasive and in situ carcinoma. There are some glands at the deep aspect of this specimen which are most likely benign but are difficult to interpret due to cautery artifact. This area does not correspond to true surgical margin.

Specimen F has skin and breast tissue, and the true surgical resection margins (inferior/lateral aspect of the specimen) are negative for invasive and in situ carcinoma. In this specimen, there is a small focus of a cauterized atypical ductal proliferation present at the superior edge of the specimen which does not represent true margin in this specimen. The differential diagnosis includes a small focus of residual DCIS, atypical ductal hyperplasia or secretory change with cytologic atypia.

GROSS DESCRIPTION

(A) LEFT AXILLARY SENTINEL LYMPH NODE #1, NOT BLUE, COUNT 9 - A single lymph node (1.5 x 1.0 x 0.5 cm). The specimen is serially sectioned and four touch preparations are performed. Entirely submitted in A1-A2.

*TP/DX: NO TUMOR IDENTIFIED.

(B) LEFT BREAST, AXILLARY CONTENTS, SHORT STITCH AT 12 O'CLOCK - A radical mastectomy specimen measures 27.0 x 18.0 x 4.0 cm with an ellipse of skin (5.5 x 5.0 cm), unremarkable nipple (0.8 x 0.8 cm) and axillary contents (9.0 x 4.0 x 3.0 cm).

The specimen is serially sectioned from medial to lateral into 16 slices. The nipple is located on slice 7. Located at the outer upper quadrant, 1 o'clock, approximately 7.0 cm from the nipple is an ill-defined firm, white mass (2.5 x 2.0 x 1.8 cm). The mass is approximately 0.6 cm from the closest superior superficial margin. Another ill-defined firm area (1.5 x 1.3 x 1.2 cm) is inferior to the first mass in the lateral aspect of the breast at approximately 2 o'clock. This lesion is approximately 2 cm from the 1 o'clock tumor, does not appear to be connected to the 1 o'clock tumor, and abuts the inferior superficial margin. There is an exophytic granular area on the skin surface at about 11 o'clock, measuring 0.5 x 0.5 x 0.2 cm. The remainder of the specimen is unremarkable. The specimen is radiographed after slicing. The suspicious areas are marked by the radiologist. Multiple lymph nodes are identified.

Representative section of the normal and tumor tissue are submitted for tumor bank.

INK CODE: Blue - superior, orange - inferior, black - deep.

SECTION CODE: B1-B14, area with circled microcalcification/superior aspect of 2.5 cm tumor, the medial aspect (B1, B5 includes superior superficial margin; B3-B4, B12-B14, inferior portion), B15-B19, 2.5 cm mass (B15, tumor with superior superficial margin; B16, tumor with deep margin; B17, tumor with superior superficial margin; B18-B19, tumor); B20, tissue between two lesions, no true margin in section; B21, 1.5 cm firm area with inferior superficial margin; B22-26, 2.5 cm tumor (B22, tumor with closest superior superficial margin, B23, tumor with deep margin, B24 with superior superficial margin, B25, with deep margin; B26, with superior superficial margin); B27, Superior aspect of 1.5 cm tumor; B28-B29: 1.5 cm lesion (B28 with superior superficial margin; B29, with deep margin) B30, inferior margin, perpendicular sections; B31-B33, 1.5 cm lesion with inferior superficial lateral margin; B34-B35, upper outer quadrant, B36-37, upper inner quadrant; B38-39, lower inner quadrant; B40,

lower outer quadrant; B41, nipple cross section; B42-B43, nipple perpendicular section; B44-B46, one lymph node serially sectioned (1.8 cm); B47, four lymph nodes; B48, one lymph node bisected (1.8 cm); B49, skin lesion.

(C) ADDITIONAL LOWER AXILLARY NODE, LEFT - Three lymph nodes ranging from 0.3 x 0.3 x 0.2 to 3.0 x 2.0 x 1.0 cm.
SECTION CODE: C1, one lymph node; C2, one lymph node trisected; C3-C5, one lymph node serially sectioned.

(D) NEW SUPEROLATERAL MARGIN, SHORT STITCH SUPERIOR, LONG STITCH LATERAL - A yellow-tan irregular fragment of adipose tissue (7.5 x 5.0 x 1.7 cm) with an attached white-tan irregular fragment of skin (6.3 x 3.0 cm). The surface of the skin displays a raised flesh colored papule (0.3 x 0.4 x 0.2 cm) that is 0.6 cm from the superior aspect of the skin. The specimen is received oriented with a short stitch at the superior margin and a long stitch at the lateral margin. The specimen is inked, serially sectioned from lateral-to-medial and no gross lesions are identified on the cut surface. Representative sections are submitted in D1-D3.

INK CODE: Blue - superior; orange - inferior; black - deep.

SECTION CODE: D1-D3, representative sections serially sectioned from lateral-to-medial.

(E) NEW SUPERFICIAL LATERAL MARGIN, STITCH TRUE MARGIN - A yellow-tan irregular fragment of fibroadipose tissue (4.5 x 4.3 x 1.1 cm). The specimen is received oriented with a stitch on the true margin. The true margin is inked in black, the specimen is serially sectioned and no gross lesions are found. Representative sections are submitted in E1-E4.

(F) NEW INFERIOR LATERAL MARGIN, SHORT STITCH SUPERIOR, LONG STITCH LATERAL - A yellow-tan irregular fragment of adipose tissue (8.9 x 4.8 x 1.9 cm) with an attached white-tan irregular fragment of skin (7.5 x 3.0 cm). The surface of the skin is grossly unremarkable. The specimen is inked, serially sectioned and no gross lesions are found. Representative sections are submitted.

INK CODE: Blue - superior; orange - inferior; black - deep.

SECTION CODE: F1-F4, representative sections serially sectioned from lateral to medial.

CLINICAL HISTORY

Left breast cancer.

SNOMED CODES

T-04050, M-85003, M-85002

"Some tests reported here may have been developed and performance characteristics determined by specifically cleared or approved by the U.S. Food and Drug Administration."

These tests have not been

Released by:

-----END OF REPORT-----